

VETERINARY SERVICES FOR HOUSEHOLD PETS

Appendix M: Remedies put to us which we are not taking forward

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The Competition and Markets Authority has excluded from this published version of the final report information which the inquiry group considers should be excluded having regard to the three considerations set out in section 244 of the Enterprise Act 2002 (specified information: considerations relevant to disclosure). The omissions are indicated by [X]. Some numbers have been replaced by a range. These are shown in square brackets. Non-sensitive wording is also indicated in square brackets.

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1. Remedies put to us which we are not taking forward

- 1.1 During our investigation, we received submissions calling on the CMA to implement a range of remedies. Where relevant, we have considered these in the sections which discuss the remedies we have decided to implement, for example where submissions have proposed different forms of these remedies. However, there were additional remedies proposed to us that were not directly related to the remedies in our package.
- 1.2 Our package of remedies is, in our view, effective in addressing the adverse effects on competition (AECs) that we have found. The additional remedies proposed to us either went beyond addressing the scope of the AECs (and in some cases were outside the scope of our investigation), or would not be an effective and proportionate way of addressing the AECs. We have therefore not included these other remedies in our package of measures. We discuss the most frequently posed of these proposals in the sections below.

Remedies to cap overall profits and prices

- 1.3 A small number of respondents to the PDR said that profits should be capped. One respondent stated that profits were excessive, and that healthcare businesses in particular should not be allowed to make excessive profits.¹ Another respondent stated that the prescription price cap did not address the markups on other treatments or services and rapid price increases on common services such as vaccines, and stated that all prices should be capped to limit the profits that could be made.²
- 1.4 In our view, the best and most effective way to deliver lower prices is through a comprehensive package of remedies that address underlying causes of the AEC itself by stimulating stronger competition and empowering pet owners to make better informed choices of providers and services. We consider that other measures to cap overall profits and prices would not be an effective or proportionate way to achieve benefits to customers. Both would likely be impracticable to implement and involve significant risks of unintended consequences.³
- 1.5 There would be significant challenges in assessing the profitability of thousands of veterinary businesses and in enforcing limits to their profits. Similarly, there would be significant challenges of implementing a price cap across all types of veterinary

¹ Individual [X] response to the PDR, p 1. Individual [X] response to the PDR, pp 3-4.

² Individual [X] response to the PDR, p 1.

³ We considered price caps on medicines prices and on cremations markups, and as detailed in part B, section 5: Medicines market opening remedies and prescription price control and section 7: Cremations. Issues with such measures apply to a more general cap on prices or profits.

services due to, the large number and variety of treatments available and in some cases the bespoke nature of the treatments.

- 1.6 Caps on overall profits or prices would have a significant risk of unintended consequences due to their potential to reduce the incentives to maintain and invest in the quality of veterinary services.

Remedies to cap the ownership of FOPs by LVGs

- 1.7 One respondent to our PDR urged us to consider both a cap on the number of FOPs an LVG may own and a limit on the percentage of FOPs that may be collectively owned by LVGs to 50% of the market, or at least no more than was currently the case.⁴ In the respondent's view this would reverse the ownership imbalance between LVGs and independents.
- 1.8 Our AEC analysis does not support measures to limit LVG ownership, and such a measure would not be proportionate. We have found there to be ineffective competition between FOPs on price and quality due to pet owners' inability to place significant demand-side constraints on veterinary businesses. A contributing factor is that some veterinary businesses do not make it sufficiently clear that their services are under common ownership or control, limiting pet owners' awareness of these matters. Our remedy in respect of transparency of ownership seeks to address this, helping pet owners to make more informed choices thereby driving the mix of business models based on the preferences of pet owners. It is better for competition to drive the range of business models compared to using regulation to determine the choices available to pet owners.
- 1.9 A remedy that restricted who can own a veterinary practice would not get at the root cause of the market-wide problems that we have found. Instead, giving pet owners information about prices and practice ownership should increase competition and put pressure on vet practices that raise prices too far above their costs.
- 1.10 We consider that measures that focus on the behaviour of businesses, and on providing information to pet owners, are the best way to make a real difference and improve competition between practices. Our remedies will help pet owners to assess how a FOP's prices compare to other practices in their area, and to know whether they are comparing FOPs owned, controlled or influenced by the same company or different ones.
- 1.11 The same respondent submitted that horizontal and vertical divestments were the only remedy which could prevent price increases which, in its view, result from

⁴ PVA response to the PDR, p 8.

concentration in the market.⁵ Linked to this, the respondent expressed concern that the CMA's merger control powers would be insufficient to address further concentration in the veterinary services market.⁶ Other respondents submitted that the CMA should consider remedies to address local concentration concerns.⁷

- 1.12 Previous CMA merger investigations in the veterinary sector have led to divestments of FOPs to address competition concerns, limiting increases in concentration in the relevant local markets. We have not found in this Market Investigation that high local concentration is sufficiently widespread to be a driver of price increases across the sector as a whole. We have also not found adverse effects on competition resulting from vertical integration. As such, it would not be appropriate to require divestments to reduce concentration in local areas as part of our package of remedies. Nevertheless, in line with its current practice, the CMA will continue actively monitoring merger activity in the veterinary sector for any relevant acquisitions that may harm competition and will take account of the analysis undertaken in this Market Investigation regarding competitive dynamics as appropriate.

Remedies to address differences in the negotiating power of FOPs and third-party retailers

- 1.13 Some of the respondents to the PDR which commented on the negotiating power of FOPs and third-party retailers with medicine manufacturers suggested that the CMA should address the disparity of negotiating power. Specifically, respondents suggested that the CMA should impose the following remedies:
- (a) a small minority said that to equalise the playing field, we should require level wholesale pricing for medicine purchases, or impose caps on rebates attained by LVGs;⁸
 - (b) a very small minority said that regulation should be changed to allow FOPs to purchase medicines from online pharmacies;⁹ and
 - (c) a handful said that medicine manufacturers should be compelled to publish list prices (including rebates) for medicines.¹⁰
- 1.14 We have not found an AEC in relation to differences in negotiating power of FOPs and third-party retailers with medicine manufacturers. This means there is no basis for imposing remedies in this area. FOPs and third-party retailers are currently

⁵ PVA response to the PDR, p 8.

⁶ PVA response to the PDR, p 17.

⁷ For example, PDR responses from: Anonymous Respondent 24 [redacted] Anonymous Respondent 1 [redacted].

⁸ For example, PDR responses from: Briar Vets Ltd, Arrowfield Veterinary Practice.

⁹ For example, Individual [redacted] PDR response.

¹⁰ For example, Treforest Veterinary Clinic PDR response.

able to negotiate competitive prices with their medicine suppliers, either directly or through membership of buying groups, and can access similar commercial terms for a given volume of committed purchases. The differences in the prices negotiated reflect efficiencies achieved through certain procurement and purchasing practices.¹¹ It is our assessment that such differences in prices between FOPs and third-party retailers is consistent with a well-functioning market.

Discounting professional fees for pet owners who purchase veterinary medicines from their FOP rather than a third-party retailer

- 1.15 FIVP suggested that we should consider rescinding Article 3 of the Supply of Relevant Veterinary Medicinal Products Order 2005.¹² This has, in FIVP's view, 'resulted in online shoppers receiving discounted professional fees that are funded by clients who purchase their medicines from the in-house practice pharmacy' and has 'restricted veterinary practices innovating and competing with on-line'.¹³
- 1.16 Article 3(3) of the Supply of Relevant Veterinary Medicinal Products Order 2005 states that '[i]t shall be unlawful for a veterinary surgeon to discriminate between a client to whom he provides prescriptions and a client to whom he does not, in—
- (a) the price he charges for a relevant veterinary medicinal product; and
 - (b) the fee he charges for supplying veterinary services other than giving a prescription.'
- 1.17 FIVP's proposal would allow FOPs to increase the professional fees paid by pet owners who request a written prescription to take advantage of the substantial savings available to them when purchasing medication from third-party retailers (usually online pharmacies). This would mean that there would be two price lists at FOPs – one with a discounted level of professional fees for pet owners who purchase medication from a FOP, another with a premium level professional fees for pet owners who request a written prescription.
- 1.18 The presence and magnitude of this difference in the professional fees charged by FOPs might discourage many pet owners from requesting a written prescription. It would also make it more difficult for pet owners to estimate the savings available to them when comparing the total costs of purchasing medication from a FOP to an online pharmacy. This would introduce a new barrier to pet owners looking to engage with the competitive offering of third-party retailers and to take advantage of the substantial savings available to them when purchasing medication outside

¹¹ This is set out in part A, section 11: Veterinary Medicines, sub-section The negotiating power of FOPs and third-party retailers when purchasing medicines.

¹² [The Supply of Relevant Veterinary Medicinal Products Order 2005, Article 3.](#)

¹³ FIVP response to the PDR, p 8.

of the FOP. This additional barrier would likely soften competition between FOPs and third-party retailers.

- 1.19 The effect of such selective discounting of professional fees would be contrary to the aims of our medicine remedies. These remedies enable pet owners to make better choices when purchasing veterinary medicines and increase the competitive pressure from online pharmacies on FOP medicine prices. Our remedies aimed at improving competition for medicines are an important part of our wider package of remedies that help reduce the overall cost to pet owners of veterinary services.
- 1.20 We therefore consider that FIVP's proposal would allow FOPs to circumvent the aim of our medicine remedies and materially reduce their effectiveness. Allowing for selective discounting of professional fees would instead maintain the status-quo where a small number of pet owners (particularly those with pets that have chronic health conditions) pay more than they need to for veterinary medicines in order to subsidise the professional fees paid by all pet owners. Therefore, we have decided not to make a recommendation to government for the Order to be amended in line with FIVP's proposal.

Remedies to address shortages of veterinary professionals

- 1.21 As noted in part A, section 2, there have been widespread issues in shortages of staff and retention of current staff. This matter was raised by a small number of respondents to our PDR. For example, one respondent to our PDR stated that the absence of a remedy to increase the number of available vets in the UK was 'a fundamental shortcoming' in our proposed remedies.¹⁴ Another respondent told us that 'Brexit has significantly increased the demand for vets to perform checks on animal and food products. In particular, the form of Brexit chosen ... has had impacts which go beyond the very narrow point acknowledged by the CMA, which is that "European vets" may have left the UK because of Brexit.'¹⁵
- 1.22 As the supply of veterinary professionals is not primarily an issue which is driven by or contributes to a competition problem,¹⁶ we did not identify it as a feature which led to our AEC findings. As such, a remedy to increase the supply of veterinary professionals would not address the AECs we have found.

A cap on out-of-hours consultation prices

- 1.23 One respondent to our PDR urged the CMA to consider a price cap on out-of-hours (OOH) consultations to ensure greater access for pet owners to affordable

¹⁴ Moore Scarrott response to PDR, p 5.

¹⁵ Individual [X] response to the PDR, p 1.

¹⁶ We have not found evidence that shortages of veterinary professionals are contributing to a competition problem.

24-hour veterinary care.¹⁷ Its view was that high prices for OOH care have been driven by local concentration of OOH ownership.

- 1.24 As noted in part A section 12, demand for OOH care is lower than for daytime veterinary care and is more expensive to provide due to uncertain workload and the higher salary of veterinary professionals that work outside of normal working hours. OOH provision is therefore inherently more concentrated than FOPs. Our local concentration analysis did not find an AEC in relation to concentration in OOH,¹⁸ hence a price cap on OOH consultation fees would not be justified or proportionate. A number of respondents to our remedies working paper and PDR noted the challenges of OOH provision, particularly in rural areas. A price cap remedy would likely make OOH provision significantly more unattractive to suppliers and worsen this already fragile situation.
- 1.25 OOH fees are included as part of our price list remedy, and this, along with our OOH contracts remedy designed to facilitate FOPs in switching OOH provider will, in our view, promote greater competition on price and quality in OOH provision.

¹⁷ PVA response to the PDR, paragraph 7.

¹⁸ This analysis found that 26% of OOH providers are located in areas where there are at least four competitors providing outsourced OOH services in close proximity to each other, 29% in areas with three competitors, and 25% in areas with two competitors. The remaining 19% of OOH providers are in areas served by only one OOH provider. See Appendix A - Local concentration analysis and barriers to entry and expansion.